SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Adjuvant trastuzumab emtansine (T-DM1)

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Adjuvant · residual invasive HER2-positive early breast cancer · NCCP 00659 · version 2 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
After preoperative treatment and surgery, residual invasive HER2-positive cancer can lead to a change to T-DM1 to reduce recurrence risk.

Schedule
T-DM1 by IV infusion every 21 days for 14 postoperative cycles, unless recurrence or unacceptable toxicity requires a change.

1. GENERIC SACT RISKS / DISCUSSION POINTS

- ☐ Purpose, expected benefit and treatment plan explained
- ☐ Treatment may be delayed, modified or stopped because of toxicity
- ☐ Unexpected, rare or serious adverse effects can occur
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Pregnancy avoidance / contraception / fertility discussed where relevant
- ☐ Bone-marrow suppression / infection risk
- ☐ Anaemia / fatigue and low platelets / bleeding
- ☐ Nausea / vomiting and bowel disturbance
- ☐ Mouth soreness / mucositis and skin changes
- ☐ Allergic / infusion reaction and other organ-specific toxicity

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Trastuzumab emtansine (T-DM1)
HER2-DIRECTED ANTIBODY WITH A CHEMOTHERAPY PAYLOAD

- ☐ Low platelets and bleeding
- ☐ Liver injury, rarely serious
- ☐ Numbness or tingling
- ☐ Heart pumping-function reduction
- ☐ Rare serious lung inflammation
- ☐ Fatigue, nausea and infusion reactions

Discuss the active phase, supportive medicines, fertility / menopause, treatment alternatives, monitoring, urgent contacts and when the plan will be reviewed.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00659 v2.

SACTCheck v0.83.8
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